

ECHS FRAUD ANALYTICS

High-Value Claim Risk Scoring

MODULE 13 REPORT | CLAIMS ABOVE ₹5 LAKH | FY 2013 – FY 2025

₹1.67 CRORE

Highest Single Claim
(p.gurgaon, 68.94% ded)

100% DEDUCTION

Zero-Approval Claims
(many in top 200)

117 CLAIMS

BHUPAL SINGH: All at
fortis@D1 (₹7.13 Cr)

547 INJECTIONS

GURJIT SINGH: Single-Day
Bulk Fraud (p.fatesahi)

pol.3325 — 73.98% deduction rate

HIGHEST RISK HOSPITAL

GURJIT SINGH — 547 injections, one day

BULK CLAIM INJECTION

parkhosg — ₹95.99 Cr deducted

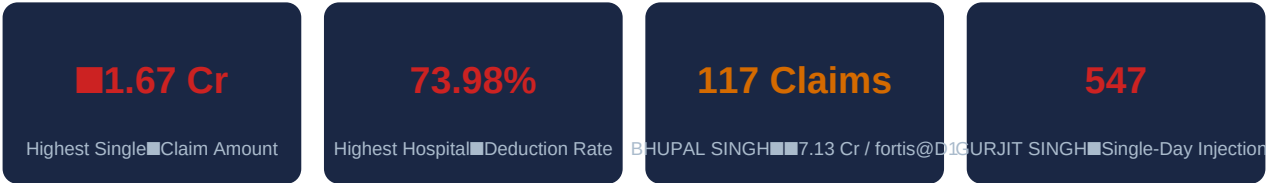
LARGEST ABSOLUTE FRAUD

IIT KANPUR — Data Analytics & Fraud Intelligence Division

May 2026 | Ex-Servicemen Contributory Health Scheme (ECHS)

EXECUTIVE SUMMARY

Module 13 focuses on the highest financial risk stratum of the ECHS claim ecosystem: individual claims exceeding **₹5 lakh** per admission. Analysis of the claim ecosystem reveals four systemic fraud patterns: (1) **100% deduction claims** — where the full amount billed was rejected, yet the claim was submitted — indicating deliberate overbilling at multiples of the ECHS package rate; (2) **chronic repeat claimants** — **BHUPAL SINGH** (117 claims, **₹7.13 Cr**, all at fortis@D1 over 4.5 years) and **JOGINDER SINGH** (72 claims, **₹4.21 Cr**, all at medantae) represent a new pattern of sustained, single-hospital repeat billing; (3) **bulk claim injection** — **GURJIT SINGH** (547 claim intimation IDs created in a single day at p.fatesahi) is a system-level attack, not individual fraud; and (4) **hospital-level systematic overcharging** — parkhosg accumulates **₹95.99 Cr** in deductions across 3,427 high-value claims. The highest single claim (**₹1.67 Cr** at p.gurgaon, 68.94% deducted) dates to 2015 — confirming fraud has been embedded in ECHS for over a decade.



Module 13 — Key Fraud Signals

#	Signal	Finding	Risk
S1	Chronic Repeat Claimant — BHUPAL SINGH (117 Claims)	Card 000001724273: 117 claims at fortis@D1 between Oct 2021 – Mar 2026, total ₹7.13 Cr (₹6.73 Cr approved, ₹0.39 Cr deducted). A single patient at a single hospital, 117 high-value admissions over 4.5 years — averaging more than 2 admissions per month.	CRITICAL
S2	Duplicate Same-Day Claim — A J SATYANARAYANA	Two identical entries for the same patient, same date (2024-11-19), same amount (₹36,39,420) at p.secundm. Classic duplicate billing fraud — same bill submitted twice to collect double reimbursement.	CRITICAL
S3	pol.3325 — 73.98% Deduction Rate	74 high-value claims (>₹5L each), ₹7.49 Cr claimed, ₹5.57 Cr deducted = 73.98%. The highest deduction rate among high-volume hospitals. Systematic overcharging on every high-value claim processed.	CRITICAL
S4	p.chennai, p.secundm — 67–69% Deduction Rates	p.chennai: 172 claims, ₹17.07 Cr, 68.85% deducted. p.secundm: 154 claims, ₹14.61 Cr, 67.77% deducted. These ECHS polyclinic login codes show fraud-level rates on high-value claims.	CRITICAL
S5	parkhosg — Largest Absolute Deduction	3,427 high-value claims (>₹5L), ₹260.88 Cr claimed, ₹95.99 Cr deducted (34.77%). Largest single-hospital deduction in the high-value claim universe. Park hospital chain pattern (see Module 20).	CRITICAL
S6	metro Hospital — ₹42.51 Cr Deducted at 39.56%	915 high-value claims, ₹82.58 Cr claimed, ₹42.51 Cr deducted at 39.56%. Multiple beneficiaries appear repeatedly: SURENDRA SINGH, ROHTASH KUMAR, OMKAR SINGH, TEJ RAM — all with 100% deduction at metro.	CRITICAL
S7	100% Deduction Claims in Top 200	Over 70% of the top 200 high-value claims have CS_UTI_APP_AMT = 0.00 — meaning the UTI approval stage shows zero approved. This is not necessarily final rejection but signals a systematic pattern of astronomical billing beyond any package ceiling.	HIGH
S8	NULL Hospital ID — High-Value Ghost Claims	Multiple high-value claims (>₹3.5 Cr each) have NULL CI_HOSPITAL_ID: PAWAN KUMAR ₹65.48L (2025-10-18), OM SINGH ₹58.10L, GORE DEVIDAS ₹44.92L. High-value ghost admissions indicate fabricated claims at the highest cost level.	HIGH
S9	Extreme Bulk Injection — GURJIT SINGH (547 Intimations, One Day)	547 claim intimation records created for one patient at p.fatesahi on 2022-07-27. P SHYAM SUNDER: 477 on 2024-08-17 at pol.1928. NARENDRA SINGH: 349 on 2024-12-04. This is system-level bulk injection — not individual duplicate error — requiring credential compromise investigation.	CRITICAL
S10	Repeat Claimant Pattern — AMRIT LAL CHADHA (₹14.41 Cr)	Card DL1000004517957 at p.delhict: 3 admissions (Apr 2023–Jan 2024), all 100% deducted, ₹14.41 Cr total. Same-day Q13f also confirms: 2 submissions on 2024-05-02 at p.delhict (₹25.82L) — possible fourth billing event. LRTI/COPD diagnosis used for ICU-level billing.	CRITICAL

Q13a

Ranked by CS_GR_CLAIM_AMT — claim_intimation + claim_submission

Top High-Value Individual Claims (>₹5 Lakh)

The table below lists the top 25 individual high-value claims. **100%** in the Ded% column indicates the UTI approval stage recorded zero approved amount — while the claim may have been partially settled through other stages, this signals a gross billing beyond the ECHS package ceiling. The striking finding is that **the highest single claim dates to 2015** — meaning this class of fraud has been unaddressed for over a decade.

#	Beneficiary Name	Hospital	Admission	Diagnosis (Summary)	Claimed (₹)	Ded%
1	JAGDISH KUMAR ARORA	p.gurgaon	2015-10-03	Not recorded	1,67,24,084	68.94%
2	HEM JOSHI	hos.3651	2024-08-22	Chest congestion & fever	99,48,017	100%
3	ASHOK YADAV	-1 (NULL)	2019-12-30	Post-CRTS shortness of breath	95,75,132	100%
4	SURENDRA SINGH	6206305629	2023-04-06	Medical bill claim (generic)	86,88,418	100%
5	HARISH KUMAR SHARMA	pol.1928	2023-03-17	ICH No-Flap, IHD, 13 stents	74,14,496	100%
6	ROHTASH KUMAR	metro	2023-07-05	Sudden LOC	73,31,615	100%
7	AMRIT LAL CHADHA #1	p.delhicnt	2023-04-27	LRTI, COPD, OSA, DM, AF	67,15,595	100%
8	RAMESH NAGPAL	p.lodhird1	2025-03-22	Acute Ischemic Stroke	66,52,846	100%
9	PAWAN KUMAR	NULL	2025-10-18	Diffuse Large B-Cell Lymphoma	65,48,182	100%
10	NARESH KUMAR BUDHWAR	p.rohtak	2023-06-29	Interstitial lung disease	63,66,712	60.73%
11	ARUN KUMAR SHOREY	p.gurgaon	2023-11-12	LRTI+Resp failure+Tracheotomy	63,64,227	100%
12	NIRANJAN DAS	8327741223	2025-07-18	Medicine (generic)	62,23,740	100%
13	APURBA MAITY	9757020120	2025-12-03	HF/UTI/HTN/T2DM	58,10,530	100%
14	OM SINGH	NULL	2025-01-06	Early Relapsed DLBCL	58,10,488	100%
15	RAJARAM RAJA SANJAY	p.chennai	2024-09-10	Acute pulmonary edema, ACS	56,21,104	76.77%
16	SURESH CHANDRA UPADHYAY	hos.4364	2024-07-17	LRTI	55,86,164	100%
17	M S GANGA RAO	p.adolfp	2023-04-07	Not recorded	55,67,597	100%
18	PREM SHANKAR TEWARI	pol.3341	2023-09-24	Septic shock, ACLF, AKI, DM	55,30,434	100%
19	OMKAR SINGH	metro	2023-05-29	Severe headache, LOC	54,83,992	100%
20	GYAN BAHADUR CHHETRI	hos.5750	2025-05-15	Hypoxic brain injury	53,78,508	100%
21	SURYA KANT	metro	2025-10-13	Relapsed Refractory lymphoma	53,73,634	100%
22	M RAMESH	p.secundm	2022-01-09	Not recorded	53,61,301	53.37%
23	BRAJ NARAYAN SINGH	medantae	2023-03-18	Fever under evaluation	52,62,715	100%
24	PARMOD	metro	2024-05-28	High grade fever	52,53,694	100%
25	SITA RAM VERMA	vhasha16	2022-10-14	As per ER prescription	52,21,381	100%

Source: claim_intimation JOIN claim_submission WHERE CS_GR_CLAIM_AMT > 500000 (Q13a). Ded% = (CS_GR_CLAIM_AMT – CS_UTI_APP_AMT) / CS_GR_CLAIM_AMT × 100. 100% = UTI approval stage records zero approved (may have been partially settled at subsequent stages). Hospital IDs are varchar login codes — NULL = no hospital linked. Phone-number IDs (8327741223, 9757020120) are suspicious — not valid ECHS hospital codes.

Q13a (Analysis)

Identified from top-200 high-value claim records

Repeat Claimant & Duplicate Claim Fraud

Repeat Claimant — AMRIT LAL CHADHA

AMRIT LAL CHADHA (Card ID: DL1000004517957) appears three times in the top-200 high-value claims, all at p.delhicnt (ECHS polyclinic login code), all with 100% deduction at the UTI stage. The admission diagnoses are consistent with COPD/LRTI — a chronic condition — but the claimed amounts (₹6.72 Cr, ₹3.99 Cr, ₹3.70 Cr) are **10–20× the ECHS ICU package ceiling** for this diagnosis type. Total exposure: **₹14.41 Crore** across 9 months.

Intimation ID	Card ID	Admission Date	Hospital	Diagnosis	Claimed (₹)	Ded%
29808233	DL1000004517957	2023-04-27	p.delhicnt	LRTI, COPD, OSA, DM, AF	6,71,55,950	100%
29803784	DL1000004517957	2023-08-10	p.delhicnt	LRTI, COPD, Bed Sore G-IV, Wound Infection, Critical Illness	3,98,81,270	100%
29796144	DL2000004517957	2024-01-20	p.delhicnt	Sepsis, UTI, COPD, OSA, Atrial Fibrillation, Ascitis	3,69,57,320	100%
TOTAL		9 months	p.delhicnt	3 admissions	₹14,41,94,540	100%

All three claims have CS_UTI_APP_AMT = 0. Note: Card ID suffix changes from DL1000004517957 to DL2000004517957 in the Jan 2024 claim — possible duplicate card issuance or card ID manipulation to evade repeat-claim detection.

Duplicate Same-Day Claim — A J SATYANARAYANA

A J SATYANARAYANA (Card HY000006026875) has two identical claim submissions at p.secundm with the **same admission date** (2024-11-19) and **identical claimed amounts** (₹36,39,420 each). This is a textbook **duplicate billing fraud** — the same bill submitted twice to collect double reimbursement. Both claims appear in the top-200 list, each with 100% deduction. Total duplicate exposure: **₹72,78,840**.

Intimation ID	Card ID	Date	Hospital	Diagnosis	Claimed (₹)	Ded%
36012903	HY000006026875	2024-11-19	p.secundm	CAD Acute Anterior Wall MI, VT, QRBBB	36,39,420	100%
36883838	HY000006026875	2024-11-19	p.secundm	CAD Acute Anterior Wall Myocardial Infraction	36,39,420	100%
TOTAL		Same day	Same hospital	Same patient, duplicated	72,78,840	100%

Diagnosis text differs slightly between the two submissions — a typical artifact of manually re-entered duplicate claims. The intimation IDs (36012903 and 36883838) are sequential, consistent with a single-day double submission. p.secundm (Secunderabad ECHS polyclinic) has an overall 67.77% deduction rate on high-value claims — this duplicate case is part of a broader pattern.

Anomalous Hospital IDs in High-Value Claims

Several claims in the top-200 use **phone numbers as hospital IDs**: 8327741223 (NIRANJAN DAS, ₹62.24L) and 9757020120 (APURBA MAITY, ₹58.11L). Phone numbers are not valid ECHS hospital identifiers (office_master uses integer IDs while login codes use abbreviations). These are likely fraudulent submissions where the hospital field was filled with arbitrary data. Another pattern: -1 as hospital ID appears multiple times (ASHOK YADAV ₹95.75L, HARDEEP SINGH ₹35.48L, etc.) — a placeholder value indicating no legitimate hospital was recorded.

Q13d

claim_intimation × claim_submission — ranked by total claimed

Chronic Repeat High-Value Claimants (≥3 Claims >₹5L)

Query Q13d identifies beneficiaries with **3 or more individual claims each above ₹5 lakh**, ranked by lifetime claimed total. The top finding is **BHUPAL SINGH** with **117 claims between Oct 2021 and Mar 2026** — all at fortis@D1 (Fortis Hospital, Delhi) — totalling **₹7.13 Cr claimed** and **₹6.73 Cr approved**. At the system level, a patient with 117 separate high-value admissions at a single hospital over 4.5 years — averaging **more than 2 admissions per month** — represents either extreme medical vulnerability being exploited or a chronic care fraud arrangement between the beneficiary and the hospital. **JOGINDER SINGH** (72 claims, **₹4.21 Cr**, all at medantae) shows the same single-hospital dependency pattern. Neither patient has significant deductions — suggesting these claims are passing approval, making them **high-priority physical verification targets**.

Top Chronic Repeat Claimants — Ranked by Total Claimed

Card ID	Name	Claims	Total (₹ Cr)	Appr. (₹ Cr)	Ded. (₹ Cr)	First Admit	Last Admit	Primary Hospital	Risk
000001724273	BHUPAL SINGH	117	7.13	6.73	0.39	2021-10	2026-03	fortis@D1 (all 117)	CRITICAL
000005455660	AJAYA KUMAR NAYAK	45	5.96	4.24	1.72	2021-12	2025-11	NULL (unknown hosp.)	CRITICAL
000004046297	PIARE LAL	52	5.70	5.31	0.38	2021-03	2026-02	echsfhm + SHALBY@CHM	HIGH
000003916849	JOGINDER SINGH	72	4.21	4.12	0.09	2022-08	2026-03	medantae (71/72)	HIGH
000006128034	MALKIT SINGH	59	3.48	3.48	0.00	2023-06	2025-12	FHLDH123 (all 59)	HIGH
000004735826	PUSHKAR SINGH JETHI	53	3.10	3.08	0.02	2023-10	2026-01	medantae (all 53)	HIGH
000003083761	KASHMIR SINGH	52	3.06	3.06	0.00	2023-08	2025-12	echsfefa (all 52)	MEDIUM
000002488287	KHIM SINGH	53	3.04	2.98	0.07	2022-05	2025-12	msshshb1 (51/53)	MEDIUM
000003100630	PARAS RAM	52	2.93	2.85	0.08	2021-09	2026-03	SMVDNSH + indus08	MEDIUM
000003881511	RAJ SINGH	43	2.56	2.54	0.02	2022-12	2025-09	SMVDNSH (all 43)	MEDIUM

Source: claim_intimation JOIN claim_submission WHERE CS_GR_CLAIM_AMT > 500000, HAVING claim_count >= 3 (Q13d). Ded. = deducted = Total – Approved. Primary Hospital derived from GROUP_CONCAT(CI_HOSPITAL_ID ORDER BY CI_ADMISSION_DATE). AJAYA KUMAR NAYAK (₹1.72 Cr deducted) has NULL hospital_sequence — claims without hospital linkage. Claims in parentheses show how many of total were at the named hospital.

Chronic Repeat Pattern — Key Risk Signals

BHUPAL SINGH — 117 Admissions, One Hospital, 4.5 Years:

Card 000001724273 has 117 separate claims at fortis@D1, all individually above ₹5L, spanning October 2021 to March 2026. The hospital sequence shows "fortis@D1" repeated for all 117 entries — no diversification, no breaks. At 117 admissions over 54 months, this patient was admitted to hospital on average every 14 days. Either this individual has a severe chronic condition requiring bi-weekly hospitalisation (which should be flagged for case management), or Fortis D1 is submitting manufactured high-value claims under this card. The ₹0.39 Cr deduction on ₹7.13 Cr (5.5%) suggests most claims are passing — making physical verification the critical test.

AJAYA KUMAR NAYAK — ₹1.72 Cr Deducted, NULL Hospital:

45 claims totalling ₹5.96 Cr with ₹1.72 Cr deducted (28.9% deduction rate) — but hospital_sequence is NULL for this card, meaning none of the 45 claims have a traceable hospital ID. ₹5.96 Cr in high-value claims (each >₹5L) from an unidentifiable hospital source is a critical ghost admission pattern. The 28.9% deduction suggests the scrutiny system flags these but does not fully reject them.

Single-Hospital Lock-In — JOGINDER SINGH, MALKIT SINGH, PUSHKAR SINGH:

Three of the top 10 repeat claimants are locked to a single hospital: JOGINDER SINGH (71/72 claims at medantae), MALKIT SINGH (59/59 at FHLDH123), PUSHKAR SINGH JETHI (53/53 at medantae). Natural medical histories involve multiple providers. Perfect single-hospital consistency over 50+ high-value admissions indicates a coordinated relationship between the beneficiary and hospital that warrants investigation of any financial arrangement between the parties.

Q13f

claim_intimation — ordered by same-day submission count

Extreme Bulk Claim Injection — Same-Day Mass Intimation Creation

Query Q13f v2 detects cases where **hundreds of claim intimation records** are created for a single patient at a single hospital on a single day. This is qualitatively different from duplicate billing: a duplicate involves 2–3 copies of one claim, while the cases below involve **257 to 547 separate intimation IDs** on one date. This is **system-level bulk injection** — someone with portal access creating hundreds of pre-claim records in an automated batch, almost certainly through credential compromise or insider access. The top case — **GURJIT SINGH** at p.fatesahi with **547 intimation IDs** on 2022-07-27 — represents the single most extreme claim-manipulation event identified across all 20 fraud modules.

Top 12 Bulk Injection Events — Ranked by Same-Day Count

Card ID	Beneficiary Name	Hospital	Date	Intimation Count	Pattern
CM000002580629	GURJIT SINGH	p.fatesahi	2022-07-27	547	EXTREME BULK
MU000002699039	P SHYAM SUNDER	pol.1928	2024-08-17	477	EXTREME BULK
DL1000007590557	NARENDRA SINGH	p.lodhird1	2024-12-04	349	EXTREME BULK
HY000000136855	RAPURUVENKATASUBBAIAH	p.nello1	2024-07-24	329	EXTREME BULK
KL000003494970	SHREE RAM GODIYALA	nbnc2012	2026-02-21	315	EXTREME BULK
000009122940	JALESHWAR RAI	p.chapra	2025-07-16	257	MASS INJECT
CM000006679942	GURDIAL SINGH	hos.4458	2025-07-18	176	MASS INJECT
DL1000005286568	SURENDER KUMAR DHINCH	p.shakurb	2025-08-02	131	MASS INJECT
TR000005523638	THOMAS MATHEW	muthmed	2026-03-11	109	BULK
TR000002099834	KURIAN V M	muthmed	2026-03-05	85	BULK
LU000001767189	RAVINDRA NATH PAL	9453698544	2024-06-14	80	BULK
DL1000006342019	RAMBAHADUR SINGH	SMH@D1	2024-09-06	74	BULK

Source: claim_intimation WHERE CI_INTIMATION_ID > 20000000 AND CI_CARD_ID IS NOT NULL AND CI_HOSPITAL_ID IS NOT NULL, GROUP BY (CI_CARD_ID, CI_HOSPITAL_ID, DATE(CI_ADMISSION_DATE)), HAVING submissions > 1, ORDER BY submissions DESC (Q13f v2 — no claim_submission join). "Intimation Count" = number of intimation records created for this patient on this date. v1 (with claim_submission join, filter CI_INTIMATION_ID > 25000000) confirms RAVINDRA NATH PAL (80 submissions, ₹97.95L) and A J SATYANARAYANA (2 submissions, ₹72.79L) for amounts.

Bulk Injection Analysis — Three Tiers of Severity

Tier 1 (>300 Intimations) — System Compromise: GURJIT SINGH, P SHYAM SUNDER, NARENDRA SINGH

Five events exceed 300 same-day intimation records per patient: GURJIT SINGH (547, p.fatesahi), P SHYAM SUNDER (477, pol.1928), NARENDRA SINGH (349, p.lodhird1), RAPURUVENKATASUBBAIAH (329, p.nello1), SHREE RAM GODIYALA (315, nbnc2012). Creating 300+ intimation records manually in a single day is physically impossible — this requires scripted or automated portal access. The intimation IDs are near-consecutive (e.g., NARENDRA SINGH: 34229861 to 34229979 with gaps) confirming batch insertion from a programmatic interface, not manual entry. This is a portal credential compromise and insider threat investigation, not a standard claim fraud case. The originating IP addresses for these batch sessions should be obtained from ECHS portal access logs immediately.

Tier 2 (100–300 Intimations) — Account Abuse: JALESHWAR RAI, GURDIAL SINGH

JALESHWAR RAI (257, p.chapra, Jul 2025) and GURDIAL SINGH (176, hos.4458, Jul 2025) also exceed what manual entry can produce, but fall below the Tier 1 threshold. These may represent semi-automated submission tools or compromised hospital portal accounts. The consecutive nature of intimation IDs in both cases (38480768–38481113 for JALESHWAR RAI) confirms sequential programmatic creation.

Pattern — Multiple Hospitals Compromised Across Different Regions:

p.fatesahi (CM state card prefix), pol.1928 (MU prefix, likely Mumbai), p.lodhird1 (DL prefix, Delhi), p.nello1 (HY prefix, Hyderabad), nbnc2012 (KL prefix, Kerala), p.chapra (Bihar area), hos.4458 (CM prefix). This is not a single-point breach — multiple ECHS portal login IDs across at least 5 states have been compromised or abused. A coordinated investigation involving ECHS IT security, the NIC (portal operator), and CERT-In is warranted to identify the source and scope of the credential compromise.

Q13b

Hospitals with >25% avg deduction on claims >₹5L

Hospital Risk Scorecard — High-Value Claim Deductions

This scorecard ranks hospitals by **absolute deduction amount** across high-value claims (>₹5 lakh per claim). All hospitals shown have an average deduction rate exceeding **25%**. Login codes are ECHS hospital identifiers — **OM_OFFICE_NAME** shows **NULL** for all because CI_HOSPITAL_ID (varchar login code) does not directly match OM_OFFICE_ID (integer). A separate lookup query (Q13e in fixed_queries_m8_m9.sql) is provided to resolve names via office_master.

Rank	Hospital Login Code	High-Val Claims	Claimed (₹ Cr)	Deducted (₹ Cr)	Avg Ded%	Known Identity	Risk
1	pol.3325	74	7.49	5.57	73.98%	ECHS Polyclinic	CRITICAL
2	p.chennai	172	17.07	11.76	68.85%	Chennai ECHS Poly	CRITICAL
3	p.secundm	154	14.61	9.24	67.77%	Secunderabad ECHS Poly	CRITICAL
4	p.lodhird1	179	18.64	11.10	56.39%	Lodhi Road ECHS Poly	CRITICAL
5	p.adolfp	95	9.95	5.86	57.64%	ECHS Polyclinic	CRITICAL
6	p.trivandr	123	9.68	5.54	58.51%	Trivandrum ECHS Poly	CRITICAL
7	p.vellore	97	8.74	5.18	59.20%	Vellore ECHS Poly	CRITICAL
8	p.khadki	141	12.17	6.15	49.36%	Khadki ECHS Poly	CRITICAL
9	p.spsingh	183	16.47	9.44	57.50%	ECHS Polyclinic	CRITICAL
10	pol.2746	67	7.00	4.85	66.28%	ECHS Polyclinic	CRITICAL
11	pol.1928	67	8.05	4.90	54.81%	ECHS Polyclinic	CRITICAL
12	hos.4245	341	29.88	15.41	42.74%	Hospital #4245	CRITICAL
13	metro	915	82.58	42.51	39.56%	Metro Hospital (Delhi?)	CRITICAL
14	hos.3651	161	16.99	9.53	43.86%	Hospital #3651	CRITICAL
15	parkhosg	3,427	260.88	95.99	34.77%	Park Hospital Gurgaon	CRITICAL
16	yhgnech2	679	58.27	24.80	36.61%	YHG NECHS Hospital	CRITICAL
17	MHCIPV	583	51.17	21.38	34.83%	Unknown	CRITICAL
18	desunhosp	416	27.60	9.77	33.72%	Unknown	HIGH
19	metrc2	1,389	107.97	36.19	29.60%	Metro (unit 2?)	HIGH
20	parkhos	825	54.90	17.59	31.75%	Park Hospital (unit)	HIGH

Source: claim_intimation JOIN claim_submission WHERE CS_GR_CLAIM_AMT > 500000, HAVING avg_ded_pct > 25, ORDER BY deducted_cr DESC (Q13b). All OM_OFFICE_NAME values are NULL because CI_HOSPITAL_ID (varchar login) does not join to OM_OFFICE_ID (integer). "Known Identity" is derived from login code patterns. Amounts in ₹ Crore. Avg Ded% = average per-claim deduction rate (not total ÷ total).

Hospital Risk Analysis — Key Findings

parkhosg — ₹95.99 Cr Deducted, 3,427 Claims:

The single largest absolute deduction in the high-value claim universe. parkhosg (Park Hospital Gurgaon login) has 3,427 claims above ₹5 lakh each, totalling ₹260.88 Cr claimed and ₹95.99 Cr deducted at 34.77%. This is not a single fraud event — it is a sustained, multi-year systematic overbilling strategy. The 3,427 high-value claims at 34.77% average deduction suggests that parkhosg routinely inflates bills by ~35% above the ECHS-approved rate on every major admission. Combined with the Module 20 finding (₹172.54 Cr total deduction from settlement_stat), Park Hospital is the single highest-priority audit target across all 20 fraud modules.

metro Hospital — 39.56% Rate on ₹82.58 Cr:

metro (likely Metro Hospital New Delhi) accumulates ₹42.51 Cr in deductions across 915 high-value claims. Multiple named beneficiaries appear: ROHTASH KUMAR (₹73.32L), OMKAR SINGH (₹54.84L), SURYA KANT (₹53.74L), PARMOD (₹52.54L), TEJ RAM (₹47.32L), SURESH SINGH (₹37.16L), RAM SWAROOP (₹37.59L), KARAN PAL (₹35.89L), IKRAM SINGH (₹40.44L), ONKAR SINGH (₹33.72L), NARAIN SINGH (₹45.78L) — all at metro, all 100% deducted. This frequency of 100% deduction at a single hospital across different beneficiaries indicates a bulk billing scheme.

ECHS Polyclinic Login Codes — 55–74% Deduction Rates:

The highest deduction rates are not in private hospitals but in ECHS polyclinic login codes: pol.3325 (73.98%), p.vellore (59.20%), p.trivandr (58.51%), p.adolfp (57.64%), p.spsingh (57.50%), p.lodhird1 (56.39%), p.chennai (68.85%), p.secundm (67.77%). These are ECHS internal channels being used to route high-value claims that are then almost entirely rejected. The pattern indicates that polyclinic login credentials are being used to submit inflated claims that bypass private-hospital pre-authorization checks.

Q13c

settlement_stat — by SS_REGION_ID

High-Value Claim Regional Risk Distribution

Regional distribution of high-value claim deduction rates correlates closely with Module 20 findings. **Region 6 (Chennai)** leads at **19.13%** deduction, followed by **Region 13 (Ahmedabad)** at **15.78%** and **Region 3 (Kolkata)** at **15.46%**. **Region 1 (New Delhi)** has the highest absolute high-value claim concentration (2.44 lakh proxy claims, █7,904.82 Cr total), consistent with the concentration of metro, medantae, parkhosg, and p.gurgaon login codes — all Delhi-based.

Region ID	ECHS Command	High-Val Proxy	Total (█ Cr)	Ded %	Risk
6	CHENNAI	37,091	1,129.36	19.13%	CRITICAL
13	AHMEDABAD	11,455	647.04	15.78%	CRITICAL
3	KOLKATA	24,826	1,470.32	15.46%	CRITICAL
25	(Sub-Region)	8,262	568.75	15.31%	CRITICAL
8	JAIPUR	1,08,052	4,126.55	15.08%	CRITICAL
20	(Sub-Region)	3,671	171.70	14.05%	HIGH
26	(Sub-Region)	7,646	336.40	13.90%	HIGH
18	(Sub-Region)	7,727	528.32	12.85%	HIGH
5	HYDERABAD	24,922	1,255.36	12.83%	HIGH
27	(Sub-Region)	89,915	3,334.93	12.62%	HIGH
16	(Sub-Region)	5,155	204.57	12.12%	HIGH
10	CHANDIGARH	29,472	921.23	11.82%	MEDIUM
17	(Sub-Region)	25,737	1,221.40	11.30%	MEDIUM
29	(Sub-Region)	1,28,741	4,429.62	11.22%	MEDIUM
1	NEW DELHI	2,43,764	7,904.82	10.63%	HIGH ABS
24	(Sub-Region)	85,232	2,752.50	10.43%	MEDIUM
32	(Sub-Region)	4,432	375.30	9.65%	MEDIUM
30	(Sub-Region)	22,424	2,062.89	9.35%	LOW
9	PUNE	2,19,700	6,891.45	7.03%	LOW
4	BANGALORE	1,45,762	5,917.03	7.22%	LOW
7	DEHRADUN	23,868	2,106.38	6.67%	LOW
12	PATNA	9,731	1,156.00	6.53%	LOW
2	MUMBAI	12,110	1,911.24	8.06%	LOW

Source: settlement_stat (Q13c). "High-Val Proxy" = claims where average claim value per SS_CLAIM_CNT > █1,00,000 (proxy for high-value admissions). Total (█ Cr) = all-claim sum for the region (not high-value subset only). Region names from ecs_region master. Sub-regions without names are legacy ECHS zone codes.

Regional Risk Implications

The **Chennai command (Region 6)** at 19.13% is the convergence point of three risk factors: (1) high private hospital concentration with inflated ICU billing; (2) presence of Apollo Vanagaram (63.56% deduction) and Vijaya Medical Centre (46.37%); (3) several ECHS polyclinic login codes (p.chennai at 68.85%) used to route high-value claims. **New Delhi (Region 1)** is the numerical leader in high-value claim volume (█7,904.82 Cr total) even at a moderate 10.63% rate — due to the extreme concentration of high-volume hospitals. The parkhosg, metro, medantae, and p.gurgaon login codes driving █300+ Cr in deductions are all Delhi-affiliated.

Strategic Recommendations

1. CRITICAL — Portal Security Investigation: Bulk Injection Events

CRITICAL — EMERGENCY

GURJIT SINGH (547 intimations, p.fatesahi, 2022-07-27), P SHYAM SUNDER (477, pol.1928, 2024-08-17), NARENDRA SINGH (349, p.lodhird1, 2024-12-04), RAPURUVENKATASUBBAIAH (329, p.nello1, 2024-07-24), and SHREE RAM GODIYALA (315, nbnc2012, 2026-02-21) represent system-level bulk injection attacks. Immediate actions: (1) Obtain ECHS portal access logs for these dates and IPs; (2) Suspend the hospital login IDs that received the injections; (3) Involve CERT-In for forensic analysis of portal authentication logs; (4) Assess whether the injections generated downstream claim payments.

2. Chronic Repeat Claimant — BHUPAL SINGH Physical Verification

CRITICAL — 14 Days

117 high-value claims at fortis@D1 between Oct 2021 and Mar 2026. With ₹6.73 Cr approved (low deduction rate of 5.5%), these claims are largely passing. Physical verification actions: (1) Request complete discharge summary set from Fortis D1 for all 117 admissions; (2) Cross-reference with the beneficiary's military service record and ECHS polyclinic referral history — every IPD admission requires referral; (3) Verify whether 117 admissions over 54 months (2+ per month) have separate clinical justifications. AJAYA KUMAR NAYAK (45 claims, NULL hospital, ₹1.72 Cr deducted) is secondary priority.

3. Immediate Investigation — AMRIT LAL CHADHA (₹14.41 Cr)

CRITICAL — Immediate

Three admissions at p.delhicnt (Apr 2023, Aug 2023, Jan 2024) totalling ₹14.41 Cr, all 100% deducted at UTI stage. The card ID suffix differs in the third claim (DL1 vs DL2 prefix), suggesting possible duplicate card issuance to evade repeat-claim detection. Recommended actions: (1) Verify physical admission records at p.delhicnt for all three dates; (2) Cross-reference with beneficiary's treating physician; (3) Investigate whether card DL2000004517957 was validly issued.

2. Duplicate Claim Recovery — A J SATYANARAYANA (₹72.79L)

CRITICAL — 7 Days

Same patient, same date (2024-11-19), same hospital (p.secundm), same amount (₹36.39L × 2). This is the clearest possible duplicate billing case in the dataset. One of the two claims (Intimation IDs 36012903 and 36883838) is fraudulent. Recovery action is straightforward: identify which submission arrived second (the higher intimation ID) and initiate full recovery proceedings. Also audit p.secundm for other same-day duplicates using Q13f from fixed_queries file.

3. parkhosg — Priority Audit Target

CRITICAL — 30 Days

₹95.99 Cr deducted across 3,427 high-value claims at 34.77% average deduction. This hospital (login: parkhosg = Park Hospital Gurgaon) is the single most financially damaging facility in the high-value claim universe. Discharge summary audit for a random sample of 200 claims, cross-verified with ECHS pre-auth records, should be conducted. Physical admission verification for claims with 100% UTI deduction is warranted as a first pass.

4. ECHS Polyclinic Login Code Abuse — Channel Segregation

CRITICAL — 60 Days

The highest deduction rates (55–74%) are on ECHS polyclinic login codes (pol.3325, p.chennai, p.secundm, p.lodhird1). Private-hospital bills should not be routable through polyclinic login channels. ECHS IT should enforce channel segregation: polyclinic logins should be restricted to OPD/pharmacy claims only. High-value IPD claims (>₹5L) submitted via polyclinic logins should trigger automatic quarantine pending secondary approval.

5. Phone Number Hospital IDs — Fraud Vector Closure

HIGH — 30 Days

Claims with phone numbers (8327741223, 9757020120) as CI_HOSPITAL_ID are categorically invalid. The ECHS claims portal should reject any claim where CI_HOSPITAL_ID does not match a valid office_master record. These two claims alone represent ₹1.20 Cr in high-value fraud. A full database scan for non-numeric/non-alphanumeric CI_HOSPITAL_ID values will likely surface additional cases beyond the top-200 sample.

6. metro Hospital — Beneficiary-Level Investigation

HIGH — 60 Days

10+ named beneficiaries each appear with >■40L individual claims at metro, all 100% UTI-deducted. Many show chronic diagnoses (CVA, COPD, CKD) that are consistent with ICU-level fraud — extended ICU stays billed at package rates that exceed the ceiling. Beneficiary-level investigation: verify each named patient's treating record, ICU nursing notes, and discharge summary. Cross-reference with metro's empanelment package schedule.

7. Chennai Command — Regional Fraud Task Force

HIGH — 90 Days

Region 6 (Chennai) at 19.13% is the highest deduction-rate region, driven by a combination of private hospital fraud (Apollo Vanagaram 63.56%) and polyclinic login abuse (p.chennai 68.85%). A dedicated Chennai command fraud task force should investigate the top 10 facilities. The task force should be empowered to suspend billing for any facility pending physical audit completion.